

Name: صباح الخير ابراهيم سيد عبدالله
National ID: 24503082101201



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

L05723824



البنك الوطني
لبنان

صباح الخير

ابراهيم سيد عبدالله

قرية بونيت

مركز النبط - الجيزة

٢٤٥٠٣٠٨٢١٠١٢٠١



٢٠٢٦/٠١

٢٥٥٠٣٠٨٢١٠١٢٠١

بنون صل

ارضية

مسلمة

انثى

الطاقة سارية حتى ٢٠٣٣/١/٠٩





Patient Name : الأستاذ / صباح الخير ابراهيم سيد
Patient No. : 24503082101201
Path No. : 435/24
Sex : Female
Age : 78 Y
Referring Doc. :
Request No. : 13003288
Request Date : 30/01/2024
Verification Date : 11/02/2024
Financial Category : أورام دار السلام - مبادرة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast lesion.

Nature of Specimen: Tru-cut biopsy, collected at 30/1/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 90% tumor cells (Allred score 8/8).
- Her2: Moderate complete membrane reaction in >10% tumor cells (score 2+) for SISH
- Ki67: 15% nuclear reaction of tumor cells.

Conclusion:

Left breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+) for SISH
- Ki67: Positive in 15% of tumor cells.

Signature

Verified by :

Dr. Heba Hasan

Printing Date : 13/02/2024 12:30:12

Page 1 of 2

Printed By : marwa mohamed yousse





Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name: الأستاذة / صباح الخير ابراهيم سيد
Patient No.: 24503082101201
Path No.: 435/24
Sex: Female
Age: 78 Y
Referring Doc.:

Request No.: 13003288
Request Date: 30/01/2024
Verification Date: 11/02/2024
Financial Category: اورام دار السلام - مبادره
Department: Out Patient

Registered by: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Prof Dr Gamal Abdelatty
Dr. Duaa Mohamed

Verified by :
Dr. Heba Hasan

Printing Date : 13/02/2024 12:30:12

Page 2 of 2

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / صباح الخير ابراهيم سيد
Patient No. : 24503082101201
Path No. : 435/24
Sex : Female
Age : 78 Y
Referring Doc :

Request No. : 13003288
Request Date : 30/01/2024
Verification Date : 03/02/2024
Financial Category : اورام دار السلام - ميله
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical data:
Left breast lesion.

Nature of specimen:
Tru cut biopsy, collected at 30/1/2024 , 1:55 pm

Gross examination
Multiple greyish white and yellowish tissue cores up to 1 cm , totally submitted.

Microscopic examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
* DCIS: Could not be detected within the limits of biopsy recieved.
* Angiolnvasion: Could not be detected within the limits of biopsy recieved.

Diagnosis:
Left breast lesion, tru cut biopsy:
Invasive duct carcinoma, Grade II .
For further IHC (ER,PR,HER2,Ki67)

Prof.Gamal Abdelatty
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

Printing Date : 13/02/2024 12:30:07

Page 1 of 1

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt



Date : 30/07/2025

Age : 80 Year(s)

Pat. ID: 46486

REVELED

Finding:

- Right breast spiculated mass 2.9 x 2 cm .
- Early interstitial pulmonary changes are seen involving both lungs manifested by subpleural lines with interstitial coarsening & fine reticular infiltrations associated with mild septal thickening.
- Few millimetric pulmonary nodules likely non specific.
- No significant pleural effusion, thickening or masses could be appreciated.
- The cardiac size is not enlarged with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Unremarkable non-enhanced appearance of the thoracic aorta and the great vessels.

ABDOMEN & PELVIS

- Right breast spiculated mass 2.9 x2 cm .
 - Early interstitial pulmonary changes are seen involving both lungs manifested by subpleural lines with interstitial coarsening & fine reticular infiltrations associated with mild septal thickening.
 - Few millimetric pulmonary nodules likely non specific.
 - No significant pleural effusion, thickening or masses could be appreciated.
 - The cardiac size is not enlarged with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
 - Unremarkable non-enhanced appearance of the thoracic aorta and the great vessels.
- ABDOMEN & PELVIS**

- The liver is still seen enlarged with in-homogeneous CT texture. ***Rather regressive course*** of the previously noted hepatic focal lesions, only one can be depicted at segment VII which still measures about 3.7 x 2.8 compared to last study 4x3cm mild intra hepatic biliary radical dilatation.
- ***Progressive course of*** Distal CBD rounded iso dense hypoenhancing lesion roughly about 2.8 x 2.5 compared to last study 1.6x1.5 cm with still noted rather the same degree of cbd as well as, common hepatic ducts and intra-hepatic biliary radicals dilatation yet with pneumobilia.
- Contracted gall bladder ?? upon faintly dense stones.
- ***Still noted*** Bulky left adrenal gland with small nodular lesion measures 0.8x1.1 cm and 2HU (adenoma) similar smaller lesion is noted within the right adrenal gland.
- ***Rather stationary course*** regarding to size and number of Multiple enlarged celiac and porto-caval lymph nodes the largest is still measures about 1.5 X 3 cm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen. Few adjacent splenules noted.

Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Tiny left renal cysts are noted.

Normal appearance of the pancreas, aorta, I.V.C.

Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

Normal appearance of the other pelvic organs.

No abdominal or pelvic masses or collections.

ned skeleton:

- The liver is still seen enlarged with in-homogeneous CT texture. ***Rather regressive course*** of the previously noted hepatic focal lesions, only one can be depicted at segment VII which still measures about 3.7 x 2.8 compared to last study 4x3cm mild intra hepatic biliary radical dilatation.
 - ***Progressive course of*** Distal CBD rounded iso dense hypoenhancing lesion roughly about 2.8 x 2.5 compared to last study 1.6x1.5 cm with still noted rather the same degree of cbd as well as, common hepatic ducts and intra-hepatic biliary radicals dilatation yet with pneumobilia.
 - Contracted gall bladder ?? upon faintly dense stones.
 - ***Still noted*** Bulky left adrenal gland with small nodular lesion measures 0.8x1.1 cm and 2HU (adenoma) similar smaller lesion is noted within the right adrenal gland.
 - ***Rather stationary course*** regarding to size and number of Multiple enlarged celiac and porto-caval lymph nodes the largest is still measures about 1.5 X 3 cm.
 - Normal size & parenchymal texture of the spleen with no focal lesions seen. Few adjacent splenules noted.
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Tiny left renal cysts are noted.
 - Normal appearance of the pancreas, aorta, I.V.C.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.
- Scanned skeleton:**

Still noted :

- Diffuse osteopenia.

- Grade I degenerative anterolateral disc lesions of L4 & L5 levels.

Opinion:

Reported case of left breast cancer shows

- **Regressive course of hepatic focal lesions**
- **Progressive course of biliary radicals dilatation down to distal CBD isodense lesion for ERCP correlation.**
- **Rather stationary course of previously noted hepatic focal lesion With contrast behavior suggestive of metastatic lesions**
- **Otherwise stationary course since the last study**

Dr. Reham Kfiedr, M.D.

